

Pregnancy & Pediatric Care Guidelines

MediCart Clinical Reference Library | Verified Guidelines

1. FDA Pregnancy Risk Categories

- **Category A:** Controlled human studies show no risk to the fetus in any trimester. (e.g. Levothyroxine, Folic Acid).
- **Category B:** Animal studies show no fetal risk, but no controlled studies in pregnant women exist, OR animal studies show risk but human studies do not. (e.g. Paracetamol, Metformin, Amoxicillin).
- **Category C:** Animal studies show adverse effects on fetus, but no human trials are available. Use only if potential benefit outweighs risk. (e.g. Ibuprofen in 1st/2nd trimester, Aspirin, Amlodipine).
- **Category D:** Positive evidence of human fetal risk based on adverse reaction data, but benefits may warrant use in life-threatening situations. (e.g. Ibuprofen in 3rd trimester - causes premature closure of ductus arteriosus).
- **Category X:** Studies in animals or humans show definite fetal abnormalities. Contraindicated in pregnancy. (e.g. Atorvastatin, Thalidomide).

2. Child & Pediatric Dosage Calculations

Pediatric doses must be calculated based on body weight (mg/kg) rather than using standard adult dosages. For example, child paracetamol dose is 10-15 mg/kg per dose. Always use a calibrated dosing syringe or spoon for liquid syrups; home teaspoons are highly inaccurate. Keep all medicines locked away out of reach of children.

3. Elderly Care and Geriatric Considerations

Geriatric patients (65+ years) frequently suffer from reduced renal and hepatic clearance. Medication doses must be titrated slowly ("start low and go slow"). Avoid polypharmacy to prevent side effects. Be highly cautious with NSAIDs (Ibuprofen) due to increased risk of acute kidney injury and gastrointestinal bleeding in older patients.